ONE Clinical Oral Hygiene Protocol

POSSIBLE F-TAG/QMS

F Tag 600

- Free of Abuse and Neglect

F Tag 676

- Maintain Abilities/ADLs

F Tag 689

- Free of Accident Hazards (Aspiration)

F Tag 692

- Nutrition/Hydration Maintenance

F Tag 697

- Pain Management

F Tag 790

- Dental Services

F Tag 880

- Infection Prevention & Control

THERAPIES

- PT
- OT
- SLP

PURPOSE OF THERAPY

Prevent infection and reduce aspiration pneumonia risk.

- Maintain adequate oral hygiene to reduce oral bacterial load.
- Prevent dental diseases and oral infections
- Reduce risk of aspiration pneumonia by decreasing bacteria that can be aspirated

Support safe and adequate nutrition and hydration

- Keep the mouth comfortable for safe eating and drinking
- Manage oral pain or dryness that may limit intake

Improve independence and participation in daily self-care

- Develop or restore ability to perform oral care tasks as part of ADLs
- Sequence steps, use adaptive equipment, or improve problem-solving for self-hygiene

GOAL EXAMPLES

SLP

- Patient will participate in an oral care routine (e.g., brushing teeth, tongue, gums, rinsing mouth) to reduce oral bacterial load prior to meals in 80% of observed opportunities.
- Patient will safely swish and spit mouthwash with minimal loss of bolus control to improve safety during oral care on 4/5 opportunities.
- Patient will allow use of suction during oral hygiene routine with caregivers on 90% of attempts to decrease risk of aspiration during oral care.
- Patient will perform thorough oral hygiene (including brushing teeth, tongue, gums, and rinsing) prior to each water intake per Free Water Protocol in 90% of opportunities to reduce bacterial load and decrease risk of aspiration pneumonia.
- Patient will sequence written steps for brushing teeth with 90% accuracy to improve follow-through with oral hygiene tasks.
- Patient will demonstrate recall of oral hygiene tasks with use of a memory aid (e.g., checklist, posted sign) with minimal cues in 80% of opportunities.

TOOLS & ASSESSMENTS

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- Oral Health Assessment Tool (OHAT)
- Risk assessment to determine Aspiration risk including:
  - Three Pillars of Aspiration
  - BOLUS Checklist
  - Risk Factors for Developing Aspiration PNA
- Clinical Swallowing Evaluation including:
  - Oral Mech/Cranial Nerve Exam
  - PO Trials
  - Cough Strength/Airway Protection
  - Secretion management
- Instrumental Swallow Assessment
  - FEEs or MBS
- Cognitive Assessment to determine ability to:
  - Recall and follow ADL routines
  - Determine need for oral hygiene via problem solving
- ADL Assessment/GG Score
- Balance and Mobility Assessment
- Use of Free Water Protocol or Ice Chip Protocol as indicated to help with oral patency
- Resistive Oral motor exercises to improve strength and ROM of musculature affecting oral health
- Use of Saliva Substitute as needed for preventing xerostomia
- Use of Sensory related swallow therapy to improve awareness of bolus and pocketed food
- Cognitive treatment from both SLP and OT focused on follow through with oral hygiene.
- Use of Balance, mobility, and strength training from PT to improve tolerance of optimal positions for oral hygiene tasks

POSSIBLE F-TAG/QMS	THERAPIES	PURPOSE OF THERAPY	GOAL EXAMPLES	TOOLS & ASSESSMENTS
Five Star Quality Measures **ADL Help Increase** - (Eating GG) - ER Visit/Hospitalization Due to infection or Pain **SNF HAI Requiring Hospitalization** - (VBP program) - Prevention of Aspiration Pneumonia by providing thorough oral hygiene		**Address swallowing safety and functional communication** - Maintain or improve oral function needed for speaking, swallowing, and social interaction **Determine and train best methods and positioning** - Assess safest and most effective oral care techniques - Optimize positioning to perform oral hygiene safely - Train caregivers on individualized methods and schedules **Respect dignity and quality of life.** - Prevent discomfort, halitosis, and embarrassment. - Support resident preferences and routines whenever possible	**OT** - Patient will perform oral hygiene tasks (e.g., brushing teeth, rinsing, denture care) with setup assistance and adaptive equipment as needed to improve independence in self-care in 80% of opportunities. - Patient will retrieve, organize, and return oral care items from a bathroom or bedside storage area with minimal cues to increase independence and safety in oral hygiene tasks. - Patient will demonstrate safe standing balance at the sink for up to 5 minutes with contact guard assist while completing oral hygiene to reduce fall risk during ADL performance. - Patient will sequence all steps of an oral care routine with the use of a visual checklist with no more than minimal verbal cues in 4/5 sessions. - Patient will demonstrate use of adaptive oral care tools (e.g., built-up handle toothbrush, long-handled mirror) with minimal verbal cues to compensate for decreased grip strength and reach in 80% of observed opportunities. - Patient will comply with daily oral hygiene routine as part of morning self-care ADL session with supervision to maximize carryover and reduce caregiver burden.	

POSSIBLE F-TAG/QMS	THERAPIES	PURPOSE OF THERAPY	GOAL EXAMPLES	TOOLS & ASSESSMENTS
			PT Patient will maintain dynamic sitting balance at the sink for 5 minutes while performing oral hygiene with supervision to reduce fall risk. Patient will tolerate standing for 3 minutes at the sink with modified independence to complete oral hygiene.	